



UNIQUE PATIENT ID: 542112

Case ID : 25010126285
 Patient Name : Mr. JITENDER KUMAR T2527
 Age/DOB/Sex : 43 Years / / Male
 Hospital Name : HCL Healthcare, Gurugram
 Physician Name : Dr. Self
 Regn Date : 30-Jun-2025 16:19
 Collection On : 30-Jun-2025 00:00
 Reported On : 01-Jul-2025 18:44
 Process AT : CORE-Gurugram.
 Ref no :
 Sample Type : Peripheral Blood
 Report Status : Interim

Thermax Health Check Up.

Parameters	Result	Unit	Biological Reference Interval
COMPLETE BLOOD COUNT			
Haemoglobin (Hb) <i>Cyanide free SLS Method</i>	15.2	g/dL	13.0 - 17.0
Hematocrit (HCT) <i>Calculated</i>	44.5	%	40 - 50
RBC Count <i>Impedance with hydrodynamic focusing</i>	L 4.37	x10⁶/uL	4.5 - 5.5
MCV <i>Calculated</i>	H 101.8	fL	83.0 - 101.0
MCH <i>Calculated</i>	H 34.8	pg	27.0 - 32.0
MCHC <i>Calculated</i>	34.2	g/dL	31.5 - 34.5
RDW CV <i>Calculated</i>	12.1	%	11.6 - 14.0
White Blood Cells and Differential Count			
Total WBC Count <i>Flowcytometry</i>	5.07	x10 ³ /uL	4 - 10
Neutrophils <i>Fluorescence flowcytometry & Microscopy</i>	60.3	%	40 - 80
Lymphocytes <i>Fluorescence flowcytometry & Microscopy</i>	26.6	%	20 - 40
Monocytes <i>Fluorescence flowcytometry & Microscopy</i>	9.7	%	2 - 10
Eosinophils <i>Fluorescence flowcytometry & Microscopy</i>	2.8	%	1 - 6
Basophils <i>Fluorescence flowcytometry & Microscopy</i>	0.6	%	0 - 2
Absolute Count			
Absolute Neutrophil Count <i>Calculated</i>	3.06	x10 ³ /uL	2.0 - 7.0
Absolute Lymphocyte Count <i>Calculated</i>	1.35	x10 ³ /uL	1.0 - 3.0
Absolute Monocyte Count <i>Calculated</i>	0.49	x10 ³ /uL	0.20 - 1.00
Absolute Eosinophil Count <i>Calculated</i>	0.14	x10 ³ /uL	0.02 - 0.50
Absolute Basophil Count <i>Calculated</i>	0.03	x10 ³ /uL	0.02 - 0.10
Platelets			
Platelet <i>Impedance with hydrodynamic focusing</i>	H 444	x10³/uL	150 - 410

Remarks: Advice: Correlation with clinical features and follow up is advised

Shivani

Dr. Shivani Sharma

DCP, DNB, DipRCPATH.
 Reg. No. 1906

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Interpretation

Note : - CBC (Syn: Full Blood Count). The red cell variables determine whether or not the patient is anaemic/ thalassemic. The white cells are often raised in infections, neutophilia in bacterial infections & lymphocytosis in viral (but not always so). Platelets (size & number) may be abnormal either as a direct or underlying blood disease, or simply reflect the presence of some other underlying pathology.

Peripheral smear examination and ESR may be added on with CBC for further evaluation of hematologic disorders.

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Thermax Health Check Up.

Parameters	Result	Unit	Biological Reference Interval
ESR			
ESR Westergren	H 20	mm/hr	0.0-10.0

Comments

1. Erythrocyte sedimentation rate (ESR) is a test that indirectly measures the degree of inflammation present in the body. The ESR is not diagnostic; it is a non-specific test that may be elevated in a number of different conditions.
2. ESR test is typically indicated for monitoring of temporal arteritis, systemic vasculitis, and polymyalgia rheumatic.
3. High ESR may be related to an inflammatory condition, such as Infection, Rheumatoid arthritis, Rheumatic fever, Vascular disease, Inflammatory bowel disease, Heart disease & Kidney disease.
4. Sometimes the ESR can be lower than normal. A slow ESR may indicate a blood disorder, such as Polycythemia, Sick cell anemia.

References

- The reference for the adult reference range is "Practical Haematology by Dacie and Lewis, 10th Edition" .

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Thermax Health Check Up.

Parameters	Result	Unit	Biological Reference Interval
Glycosylate Hemoglobin (HbA1C)			
Glycated Hemoglobin (HbA1c) <i>High Performance Liquid Chromatography</i>	4.4	%	Nondiabetic: < 5.7 Prediabetics: 5.7- 6.4 Diabetics: >or = 6.5
Estimated Average Glucose (eAG) <i>Calculated</i>	79.3	mg/dL	

Comments

1. Glycated hemoglobin (GHb) has been firmly established as an index of long term blood glucose concentrations and as a measure of the risk for the development of complications in patients with diabetes mellitus.
2. Formation of GHb is essentially irreversible, and the concentration in the blood depends on both the life span of the red blood cell (average 2-4 weeks) and the blood glucose concentration.
3. This test is not suitable for patient with recent blood loss, hemolytic anemia, hemoglobinopathy, polycythemia or splenectomy.

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Reg. No. 1906

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PhD. Medical Biochemistry



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Thermax Health Check Up.

Parameters	Result	Unit	Biological Reference Interval
Bilirubin Indirect			
Bilirubin Indirect <i>Calculated</i>	0.27	mg/dL	0.1-1.0
Bilirubin Total			
Bilirubin Total <i>Diazo Method</i>	0.475	mg/dL	0.2-1.0
Direct Bilirubin			
Bilirubin, Direct <i>Diazo Method</i>	0.210	mg/dL	0.0-0.2

Comments

1. Conjugated (direct) bilirubin is elevated more than unconjugated (indirect) bilirubin in Viral hepatitis, Drug reactions, Alcoholic liver disease.
2. Conjugated (direct) bilirubin is also elevated more than unconjugated (indirect) bilirubin when there is some kind of blockage of the bile ducts like in Gallstones getting into the bile ducts, tumors & Scarring of the bile ducts.

References

Tietz Textbook of Clinical Chemistry. Edited by CA Burtis, ER Ashwood. Philadelphia, WB Saunders Company, 1994

Dr. Shivani Sharma

DCP, DNB, DipRCPath.
Reg. No. 1906

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Thermax Health Check Up.

Parameters	Result	Unit	Biological Reference Interval
Protein Total			
Protein Total <i>Biuret method</i>	8.0	g/dL	6.4-8.2

Interpretation

- Protein in the plasma is made up of albumin and globulin.
- Higher than normal levels may be due to: Chronic inflammation or infection, including HIV and hepatitis B or C, Multiple myeloma, Waldenstrom's disease.
- Lower than normal levels may be due to: Agammaglobulinemia, Bleeding (hemorrhage), Burns, Glomerulonephritis, Liver disease, Malabsorption, Malnutrition, Nephrotic syndrome, Protein losing enteropathy etc.

Alkaline Phosphatase (ALP)

Alkaline Phosphatase (ALP) <i>Para-nitrophenyl Phosphate</i>	114.0	U/L	40 - 129
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Comments:

- Alkaline phosphatase levels are seen in Biliary obstruction, Osteoblastic bone tumors, osteomalacia, hepatitis, Hyperparathyroidism, Leukemia, Lymphoma, Paget's disease, Rickets, Sarcoidosis etc.
- Lower than normal ALP levels seen in Hypophosphatasia, Malnutrition, Protein deficiency, Wilson's disease.
- Tissues with higher amounts of ALP include the liver, bile ducts and bone.

References:

Tietz NW: Textbook of Clinical Chemistry. Third edition, Edited by Burtis and Ashwood. Philadelphia, PA, WB Saunders Company, Philadelphia, 1999.

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Aspartate Aminotransferase (SGOT)

AST (SGOT) <i>IFCC with pyridoxal phosphate</i>	33.5	U/L	10 - 50
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Comments

1. SGOT is found in the liver, heart, skeletal muscle, kidneys, brain, and red blood cells, and it is commonly measured clinically as a marker for liver health.
2. SGOT levels increase during chronic viral hepatitis, blockage of the bile duct, cirrhosis of the liver, liver cancer, kidney failure, hemolytic anemia, pancreatitis, hemochromatosis.
3. SGOT levels may also increase after a heart attack or strenuous activity.

References

- Tietz Textbook of Clinical Chemistry. Edited by CA Burties, ER Ashwood. Philadelphia, WB Saunders Company, 1994.

Alanine Aminotransferase (ALT)

ALT (SGPT) <i>IFCC with pyridoxal phosphate</i>	35.8	U/L	10 - 50
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Comments:

1. Alanine Aminotransferase (SGPT) is found mainly in the liver, but also in smaller amounts in the kidneys, heart, muscles, and pancreas.
2. SGPT levels increase during acute hepatitis, sometimes due to a viral infection, ischemia to the liver, chronic hepatitis, obstruction of bile ducts, cirrhosis.

References:

Tietz Texbook of Clinical Chemistry. Edited by CA Burtis, ER Ashwood Philadelphia, WB Saunders Company, 1994

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Thermax Health Check Up.

Parameters	Result	Unit	Biological Reference Interval
A/G Ratio			
Protein Total <i>Biuret method</i>	8.0	g/dL	6.4-8.2
Albumin <i>Bromocresol Green Colorimetric</i>	4.7	g/dL	3.5 - 5.2
Globulin <i>Calculated</i>	3.3	g/dL	2.5-3.5
Albumin:Globulin Ratio - A:G Ratio <i>Calculated</i>	1.42	Ratio	1.0 - 2.0

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Thermax Health Check Up.

Parameters	Result	Unit	Biological Reference Interval
Kidney Function Test			
Urea Serum <i>Kinetic test with urease</i>	18.1	mg/dL	12.6-49.2
Blood Urea Nitrogen (BUN) <i>Urease</i>	L 8.5	mg/dL	8.9-20.6
Creatinine Serum <i>Jaffé method</i>	L 0.59	mg/dL	0.70-1.20
eGFR <i>Calculated</i>	123.5	mL/min/1.73 m2	>90 ml/min/1.73m2
Uric Acid Serum <i>Uricase</i>	4.8	mg/dL	3.5-7.2
Calcium <i>NM-BAPTA</i>	9.9	mg/dL	8.5-10.1
Sodium (Na+) <i>Ion Selective Electrodes</i>	137.30	mmol/L	136.0-146.0
Potassium (K+) <i>Ion Selective Electrodes</i>	4.28	mmol/L	3.4-5.10
Chloride (Cl-) <i>Ion Selective Electrodes</i>	99.6	mmol/L	98-106

COMMENT :

- Causes of increased levels of BUN: a) Pre renal: High protein diet, Increased protein catabolism, GI hemorrhage, Cortisol, Dehydration, CHF Renal b) Renal: Renal Failure c) Post Renal: Malignancy, Nephrolithiasis, Prostatism
- Creatinine higher than normal level may be due to: Blockage in the urinary tract, Kidney problems such as kidney damage, failure, infection, reduced blood flow; Loss of body fluid (dehydration); Muscle problems such as breakdown of muscle fibers; Problems during pregnancy such as seizures (eclampsia) or high blood pressure caused by pregnancy (preeclampsia)
- Serum uric acid levels are increased in gout, renal failure, inherited metabolic disorders, excess dietary purine intake, increased nucleic acid turnover (e.g. leukemia, myeloma, radiotherapy, chemotherapy, trauma), psoriasis, preeclampsia and alcohol consumption.
- Electrolytes (Na, K and Cl) play an important role in water homeostasis, maintenance of pH, regulation of heart and muscle function, as well as act as co-factors for enzymes. The kidneys help to maintain electrolyte concentrations by regulating their concentrations in the body.

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Parameters	Result	Unit	Biological Reference Interval
Lipid Profile Lite			
Cholesterol <i>CHO-POD</i>	157.0	mg/dL	Desirable cholesterol level < 200 Borderline cholesterol 200-239 High cholesterol = 240
Triglyceride <i>GPO - POD METHOD</i>	106.0	mg/dL	Normal: <150 Borderline: 150-199 High: 200-499 Very High: >500
HDL Cholesterol <i>Homogeneous Enzymatic (CHOD-POD) Colorimetric</i>	51.2	mg/dL	Major risk factor for heart disease <40 Negative risk factor for heart disease ≥60
LDL Cholesterol <i>Calculated</i>	84.60	mg/dL	Optimal: <100 Near or above optimal: 100-129 Borderline high: 130-159 High: 160-189 Very high: ≥190
VLDL Cholesterol <i>Calculated</i>	21.2	mg/dL	< 30
Cholesterol/HDL Ratio : <i>Calculated</i>	L 3.1		Low Risk: 3.3-4.4 Average Risk: 4.5-7.0 Moderate Risk: 7.1-11.0 High Risk: >11.0
LDL/HDL Ratio <i>Calculated</i>	1.65		Desirable/Low Risk: 0.5-3.0 Borderline/Moderate Risk: 3.1-6.0 High Risk: >6.0

Comments

- Serum cholesterol & its lipoprotein carriers (LDL, VLDL & HDL) are related to atherosclerotic cardiovascular disease. LDL-C is the dominant form of atherogenic cholesterol but HDL-C is not. VLDL is the chief carrier of triglycerides & VLDL-C is also atherogenic.
- The combination of LDL-C and VLDL-C is called non-HDL-C and is more atherogenic than either lipoprotein alone.
- HDL-C is sometimes called the "good" cholesterol because it helps to carry away LDL-C, thus keeping arteries open and blood flowing more freely. HDL-C is inversely related to the risk of cardiovascular disease. It increases following regular exercise or with oral estrogen therapy. Decreased levels are associated with obesity, stress, cigarette smoking and diabetes mellitus.
- High triglyceride levels are associated with several factors including being overweight, eating too many sweets, drinking too much alcohol, smoking, being sedentary or having diabetes with elevated blood sugar levels.

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Thermax Health Check Up.

Parameters	Result	Unit	Biological Reference Interval
Thyroid Function Test			
T3, Total <i>ECLIA</i>	1.18	ng/mL	0.87 - 1.78
T4, Total <i>Electrochemiluminescence immunoassay</i>	6.59	µg/dL	6.09 - 12.23
Thyroid Stimulating Hormone, TSH <i>Electrochemiluminescence immunoassay</i>	0.75	µIU/mL	0.27-4.20

Comments

- TSH Levels are subjected to circadian variation, rising several hours before the onset of sleep, reaching peak levels between 11 pm and 6am & nadir concentrations during the afternoon. Diurnal variation in TSH levels is approx. 50% +/- hence, time of the day can influence the measured serum concentration.

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DCP, DNB, DipRCPath.
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Thermax Health Check Up.

Parameters	Result	Unit	Biological Reference Interval
Vitamin B12, Cyanocobalamin			
Vitamin B12 <i>ECLIA</i>	L 210.0	pg/mL	232 - 1245

Comments

- Vitamin B12 is a water soluble vitamin required for the maturation of red blood cells and general somatic cell metabolism.
- Decreased Levels: Lack of Intrinsic factors, intestinal malabsorption, loss of ingested Vitamin B12 due to fish tapeworm.
- Increased Levels: Chronic renal failure, liver disease, congestive heart failure, acute & chronic myeloid leukemia.

Vitamin D 25 Hydroxy

Vitamin D 25 Hydroxy <i>ECLIA</i>	L 8.53	ng/mL	Normal : 20-100 Sufficient : 30-100 Insufficiency : 20-29 Deficiency : < 20 Toxicity : > 100
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Comments

Vitamin D3 (Vitamin D 25 Hydroxy):

- Vitamin D is essential for bone health. In children severe deficiency leads to rickets. Milder degrees of insufficiency are believed to cause reduced efficiency in the utilization of dietary calcium.
- Vitamin D deficiency also causes muscle weakness. In conjunction with other clinical data, the results may be used as an aid in the assessment of bone metabolism.
- Elevated PTH levels along with vitamin D deficiency in adults can result in osteomalacia, increased bone turnover, reduced bone mass and risk of bone fractures.

References

- Tietz Textbook of Clinical Chemistry and Molecular Diagnostics, edited by Carl A Burtis, Edward R. Ashwood, David E Bruns, 4th edition, Elsevier publication, 2006, 1920 1926

Pending Services

Urine Routine Examination & Microscopy (CC)

Dr. Shivani Sharma

DCP, DNB, DipRCPATH.
Reg. No. 1906

Dr. Onkar Nath Tiwari

PhD. Medical Biochemistry



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CORE Diagnostics (Central Reference Lab) - Gurugram
406, Udyog Vihar, Phase III, Gurugram, Haryana - 122016
T.: +91 8882899999

CORE Diagnostics Satellite Lab - New Delhi

C-13, 1st Floor, Green Park Extension, New Delhi - 110016 | T.: 011 46269604
67, Hargobind Enclave, New Delhi - 110092 | T.: +91 9318333855
H64, Block H, Bali Nagar, New Delhi - 110015 | T.: 011-41504265

CORE Diagnostics Satellite Lab - Bhubaneswar

Plot No. - 249, Near Police Academy, AIIMS Nagar,
Patrapada, Bhubaneswar - 751019
T. : +91 9599401177

CORE Diagnostics Satellite Lab - Lucknow

J.S. Tower, Plot No. - K-702, Sector K, Ashiyana, Near Raj
Luxmi Sweets, Lucknow, Uttar Pradesh - 226012
T.: 0522- 4580564, +91 92050 30164

CORE Diagnostics Satellite Lab - Bangalore

1st Floor, KMK Tower, 142 KH Road,
Bangalore - 560027
T.: 080-22244777, +91 8951870077

CORE Diagnostics Satellite Lab - Guwahati

Ground Floor, Honuram Boro Path, Shubham Velocity, GS Road,
Dispur, Kamrup Metropolitan, Guwahati, Assam - 781005
T.: +91 9318333749

CORE Diagnostics Satellite Lab - Hyderabad

S. No.155, Mahalakshmi Trade Center, 2nd Floor, Thokatta -Village,
Diamond Point Road, Bowenpally, Secunderabad,Telangana-500009
T.: +91 9100122608



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 Sample Type : Urine
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Thermax Health Check Up.

Parameters	Result	Unit	Biological Reference Interval
Urine Examination Routine			
Physical Examination			
Colour	Pale Yellow		Pale Yellow
Volume	10	ml	.
Appearance	Clear		Clear
Chemical Examination			
Blood <i>Peroxidase</i>	Absent		Negative
Protein <i>Error-of-indicator</i>	Negative		Negative
Glucose <i>Glucose oxidase</i>	Negative		Negative
Ketones <i>Sodium nitroprusside</i>	Negative		Negative
Bilirubin <i>(Dichloroaniline diazonium salt)</i>	Negative		Negative
Urobilinogen <i>Diazonium Salt</i>	0.5		Normal
Nitrite <i>Amino sulphanilamide</i>	Negative		Negative
pH <i>pH indicator</i>	8.9		5 - 9
Specific Gravity <i>pKa Change</i>	1.015		1.005 - 1.030
Leukocyte Esterase <i>Esterases</i>	Negative		Negative
Microscopic Examination			
Pus Cells <i>Microscopy</i>	1-4	/HPF	0 - 5
Red Blood Cells (RBC) <i>Microscopy</i>	Nil	/HPF	Nil
Epithelial Cells <i>Microscopy</i>	1-3	/HPF	0 - 5
Cast <i>Microscopy</i>	Nil	/HPF	Nil
Crystals <i>Microscopy</i>	Nil	/HPF	Nil
Bacteria <i>Microscopy</i>	Absent	/HPF	Absent / Present
Yeast cells <i>Microscopy</i>	Nil	/HPF	Nil

Shivani

Dr. Shivani Sharma

Subhradeep

Dr. Subhradeep
Majumder

DCP, DNB, DipRCPath.

MD, DNB, Reg No. - 52661



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Pending Services

Stool Examination (CC)

Dr. Shivani Sharma

DCP, DNB, DipRCPath.

**Dr. Subhradeep
Majumder**

MD, DNB, Reg No. - 52661

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CONDITIONS OF REPORTING

1. The tests are carried out in the lab with the presumption that the specimen belongs to the patient named or identified in the bill/test request form.
2. The test results relate specifically to the sample received in the lab and are presumed to have been generated and transported per specific instructions given by the physicians/laboratory.
3. The reported results are for information and are subject to confirmation and interpretation by the referring doctor.
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CORE Diagnostics (Central Reference Lab) - Gurugram

406, Udyog Vihar, Phase III, Gurugram, Haryana - 122016

T.: +91 8882899999

CORE Diagnostics Satellite Lab - New Delhi

C-13, 1st Floor, Green Park Extension, New Delhi - 110016 | T.: 011 46269604

67, Hargobind Enclave, New Delhi - 110092 | T.: +91 9318333855

H64, Block H, Bali Nagar, New Delhi - 110015 | T.: 011-41504265

CORE Diagnostics Satellite Lab - Bhubaneswar

Plot No. - 249, Near Police Academy, AIIMS Nagar,
Patrapada, Bhubaneswar - 751019

T. : +91 9599401177

CORE Diagnostics Satellite Lab - Bangalore

1st Floor, KMK Tower, 142 KH Road,
Bangalore - 560027

T.: 080-22244777, +91 8951870077

CORE Diagnostics Satellite Lab - Lucknow

J.S. Tower, Plot No. - K-702, Sector K, Ashiyana, Near Raj
Luxmi Sweets, Lucknow, Uttar Pradesh - 226012

T.: 0522- 4580564, +91 92050 30164

CORE Diagnostics Satellite Lab - Guwahati

Ground Floor, Honuram Boro Path, Shubham Velocity, GS Road,
Dispur, Kamrup Metropolitan, Guwahati, Assam - 781005

T.: +91 9318333749

CORE Diagnostics Satellite Lab - Hyderabad

S. No.155, Mahalakshmi Trade Center, 2nd Floor, Thokala Village,
Diamond Point Road, Bowenpally, Secunderabad, Telangana-500009

T.: +91 9100122608

MEDICAL EXAMINATION FORM

SERIAL NO:

DATE: 30 - June- 2025

NAME	Jitender Kumar	GENDER	Male
AGE	43	EMPLOYEE ID	

GENERAL EXAMINATION

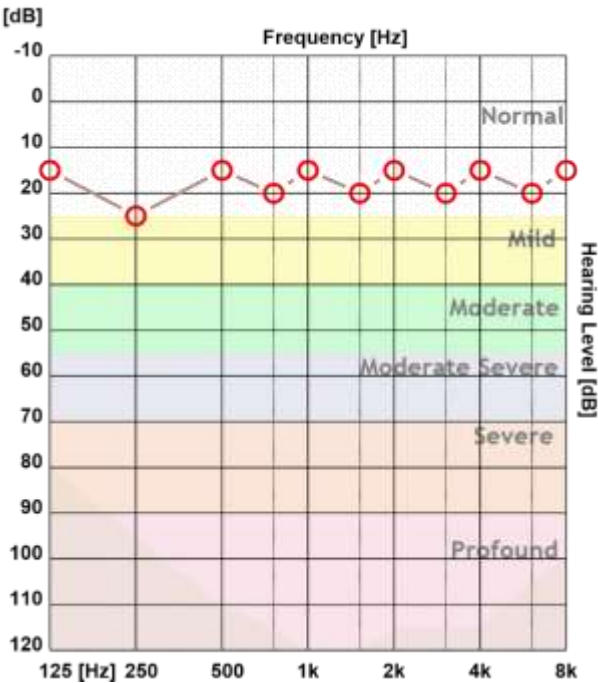
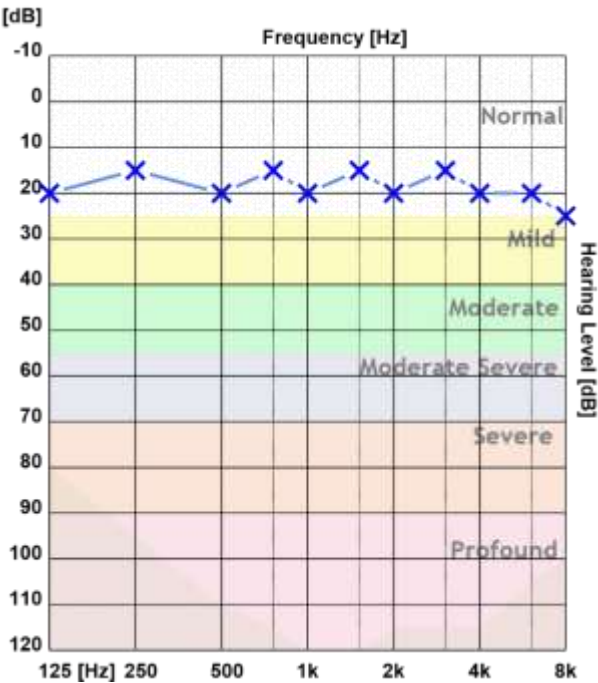
WEIGHT (kgs)	75
HEIGHT (cms)	5'4
BLOOD PRESSURE (mmHg)	115/79
BMI	
SKIN EXAMINATION:	Normal
EYE	6/6 (Normal Vision)

SIGNATURE WITH SEAL:


Dr. Abhilash Rout
MBBS, MD
Regd. No-16087106(OM)
Name and signature with Seal
Of Registered Medical Practitioner

Name	Jitender Kumar	Gender	Male
Age	43	Req Date and Time	28-June-2025

125



Hz

250 Hz 500 Hz 750 Hz 1000 Hz 1500 Hz 2000 Hz 3000 Hz 4000 Hz 6000 Hz 8000 Hz

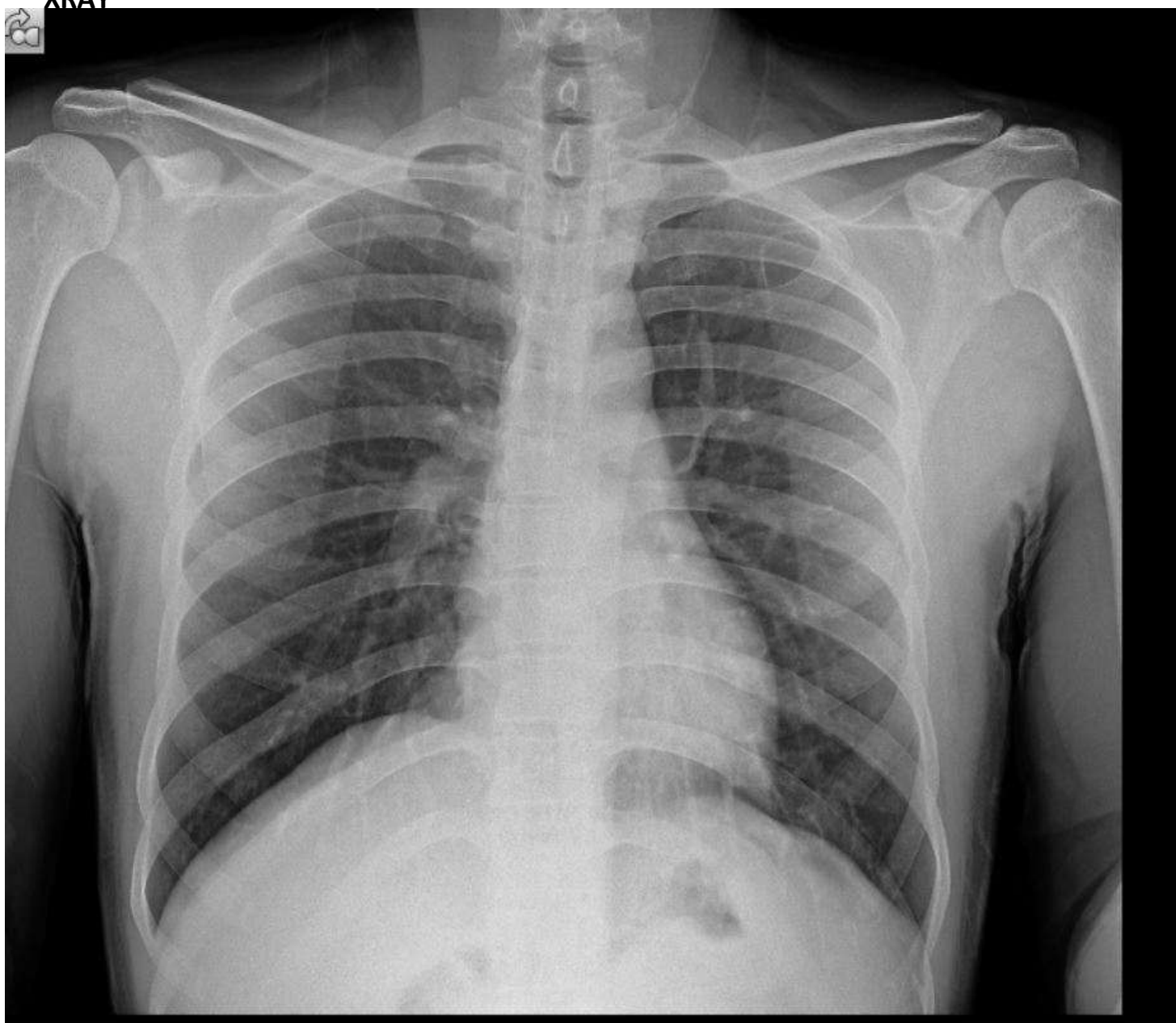
X - Air Left	20	15	20	15	20	15	20	15	20	20	25
O - Air Right	15	25	15	20	15	20	15	20	15	20	15
> - Bone Left											
< - Bone Right											

	Average	High	Mid	Low
AIR Left	18.64 dB	20.00 dB	18.33 dB	17.50 dB
AIR Right	17.73 dB	17.50 dB	16.67 dB	18.75 dB

Clinical Notes : Not Found

Dr. Diptendu Kumar Biswas
 Dr. Diptendu Kumar Biswas
 MBBS,DMRD.[Consultant Radiologist]
 RegistrationNo:61828[WEMC]

XRAY





SEBA

Diagnostic Centre

RichTextEditor, myeditor

Chanda Jamuria Road
Near Petrol Pump
Dist. Paschim Bardhaman, W.B.
Mobile : 9933903490, 9153061364
Office : 9932668847
Email : sebadiagnosticcentre4@gmail.com

Patient ID: 03923	Date: 27/06/25
Name: MR. JITENDRA KUMAR	Age: 43Y
Gender: M	Modality: CR
Ref. By Dr.: SELF.	Test:

X-RAY-CHEST[PA]VIEW

- Lung fields show normal lucency.
- The cardiac shadow is normal in contour.
- Domes & costophrenic angle are normal
- Mediastinum central.
- Rib cage/ soft tissues are normal.

IMPRESSION: Normal study.

--Suggested further clinical correlation & follow-up.

Diptendu Kumar Biswas

Dr. Diptendu Kumar Biswas
MBBS, DMRD. [Consultant Radiologist]
Registration No: 61828 [WBMC]

Disclaimer: It is an online interpretation of medical imaging based on clinical data. All modern machines/procedures have their own limitations. If there is any clinical discrepancy, this investigation may be repeated or reassessed by other tests. Patient's identification in online reporting is not established, so in no way this report can be utilised for any medico-legal purpose.